Mood Disorders: Depressive and Bipolar Spectrums

Psychiatry, Clinical Lecture | Dr. Hussain M. Khrad (Consultant Psychiatrist, KFSHRC-Jeddah) | Source: Drive file `6870348` (70 slides)

> **Legend:** ⭐ = high-yield · *[added]* = not on slides. Images marked "Slide N" come from the lecture. All MCQ answers and the case model answer come from the lecture's own answer slides.
>
> 🔗 **Bridge from earlier lectures:**
>
> - **History Taking / MSE:**
> - mood (subjective, the climate) vs affect (objective, the weather)
> - suicide assessment (previous attempt = strongest predictor)
> - collateral is essential because **hypomania is rarely volunteered**
> - **Personality Disorders: BPD vs bipolar** (hours + interpersonal trigger vs days–weeks + ↓ need for sleep).
> - **Psychotic Disorders:**
> - **schizoaffective 2-week rule**
> - **catatonia is commonest in mood disorders**
> - nihilistic delusions (**Cotard**) in psychotic depression
> - **Endocrine:**
> - **hypothyroidism** (Thyroid I) and **Cushing's/Addison's** (Adrenal I/II) mimic depression
> - **hyperthyroidism/Cushing's** precipitate mania
> - **HPA-axis overactivity** and dexamethasone non-suppression (Adrenal I) appear again

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1. Title / Objectives

Domain	Objective	Likely tested as
Knowledge	Sadness vs depressive disorder	≥ 2 weeks, out of proportion, impairing
Knowledge	⭐ Depressive and bipolar disorders + DSM-5-TR criteria	SIG-E-CAPS; DIG FAST; mania vs hypomania
Knowledge	Monoamine, HPA-axis, neuroplastic models	Why a 4–6-week delay
Knowledge	⭐ **Specifiers that change management**	Atypical → MAOIs; psychotic → antipsychotic/ECT; peripartum
Skill	⭐ **Screen every depressed patient for past mania**	Bipolar II case
Skill	Structured suicide risk assessment	Previous attempt
Skill	⭐ Exclude organic mimics	Hypothyroidism case; alcohol case
Skill	Separate **grief, adjustment disorder, MDD**	Worthlessness/guilt = MDD
Skill	Match treatment intensity to severity	Stepped care
Values	Depression is a **systemic, potentially lethal** illness; ask about suicide directly; cultural frame; family as an ally; honest prognosis	—

Why it matters (slide 3): mood disorders are the **commonest psychiatric presentation in every specialty**, and suicide risk is assessed by **whoever is in front of the patient**.

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2. Main Content (Lecture Order)

2.1 Sadness vs Depression; the Mood Disorder Landscape (Slides 4–6)

Sadness	Depression (disorder)
Normal, universal emotion	**Pervasive low mood and/or anhedonia**
Proportionate to a trigger; transient	**Persistent (≥ 2 weeks)** and **out of proportion**
Self-esteem and function preserved	**Impairs** social and occupational function
Resolves without treatment	Often needs treatment

Mood = a sustained, pervasive emotional **climate** (vs brief, reactive affect). Mood disorders are primary disturbances of that climate.

Depressive spectrum (one pole)	Bipolar spectrum (both poles)
MDD	**Bipolar I**
Persistent depressive disorder (dysthymia)	**Bipolar II**
PMDD	**Cyclothymic disorder**
DMDD (disruptive mood dysregulation)	Substance/medication- and medical-condition-induced
Substance/medication- and medical-condition-induced	—

⭐ Placing the patient in the right family changes treatment completely: antidepressant monotherapy can destabilise a bipolar patient.

Saudi context (slide 6):

- depression and anxiety are among the commonest national disorders; the **Saudi National Mental Health Survey** shows substantial lifetime prevalence, **mostly untreated**
- young population → large disability burden
- **stigma and somatic presentation** ("aches all over", fatigue) delay help-seeking
- primary-care integration under **Vision 2030**
- family context is central

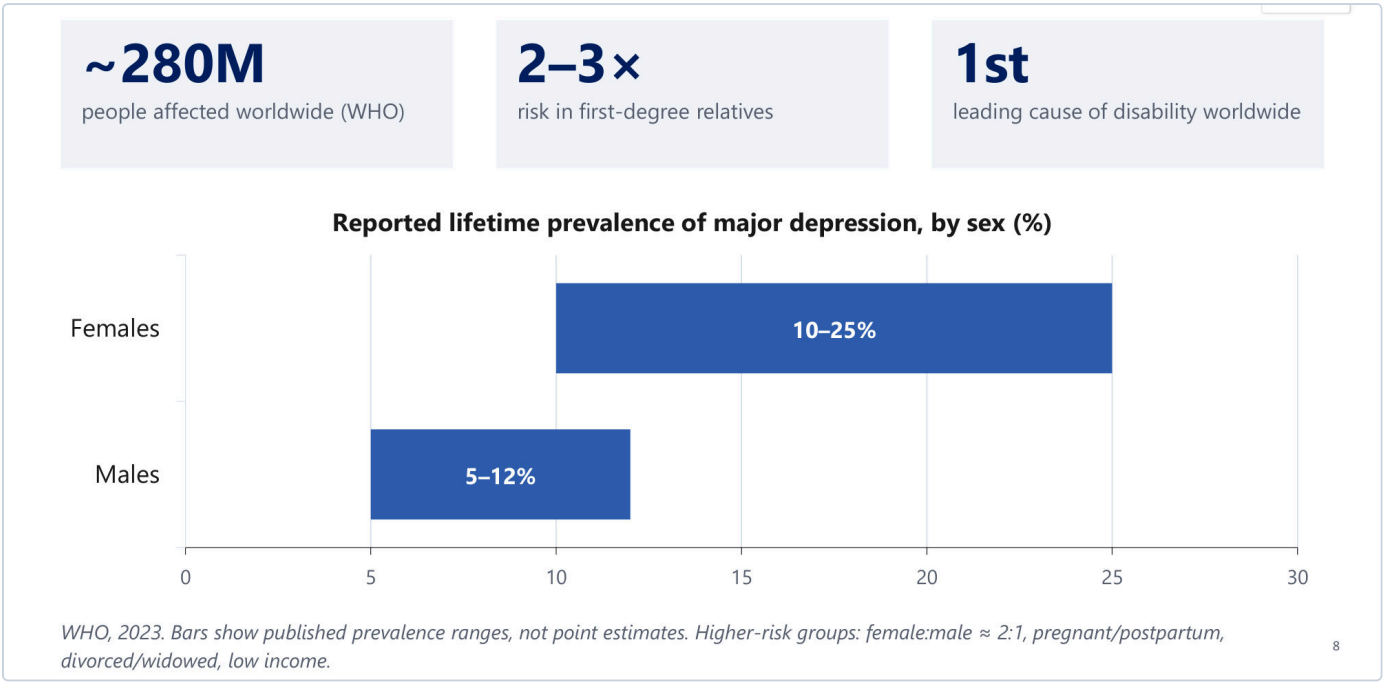
2.2 Epidemiology of Depression (Slide 8) ⭐

Figure	Value
People affected worldwide (WHO)	**~280 million**
Global disability ranking	⭐ **1st leading cause of disability**
Risk in first-degree relatives	**2–3×**
Lifetime prevalence: **males**	**5–12%**
Lifetime prevalence: **females**	**10–25%**
F:M	⭐ **≈ 2:1**

Higher-risk groups:

- women

- **pregnant/postpartum**
- **divorced/widowed**
- low income



Slide 8: Depression epidemiology

2.3 Aetiology and Neurobiology (Slides 9–10)

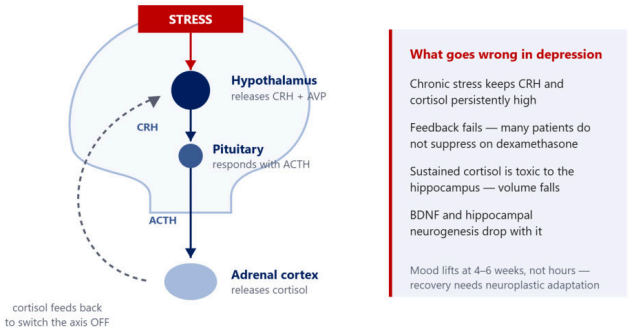
Model	Content
Neurotransmitter	**Monoamine** dysregulation: serotonin, noradrenaline, dopamine
Genetic	Heritable; first-degree relatives; twin concordance. ⭐ **Heritability ~40% (MZ > DZ)**
Stress-diathesis	Latent vulnerability unmasked by life stress
Neuroendocrine	⭐ **HPA-axis overactivity, raised cortisol**; thyroid axis
Neuroanatomic/neurotrophic	Limbic–prefrontal dysfunction; ⭐ **↓ BDNF and hippocampal volume**
Sleep & psychological	⭐ **Shortened REM latency**; **negative cognitive triad**; **learned helplessness**

HPA mechanism (slide 10):

1. chronic stress → persistently high **CRH and cortisol**
2. **feedback fails**: many patients **don't suppress on dexamethasone**
3. sustained cortisol is **toxic to the hippocampus** (volume falls)
4. **BDNF and neurogenesis drop**

⭐ **Why the delay?** Monoamines rise in **hours**, but mood lifts in **4–6 weeks**: recovery needs **receptor and neuroplastic adaptation**.

Chronic stress drives the HPA axis — and excess cortisol is toxic to the hippocampus.



Why the delay? Monoamines rise in hours, but mood lifts in 4–6 weeks — recovery depends on slower receptor and neuroplastic adaptation. Heritability ~40% (MZ > DZ).

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Slide 10: HPA axis in depression

2.4 Why Treat Depression? (Slide 11)

- **Systemic harm:**
  - ⭐ **independent risk factor for MI and stroke**
  - worsens **diabetes** and chronic disease
  - drives ED visits and admissions
- **Mortality:**
  - ⭐ **40–60% of completed suicides involve a depressed patient**
  - leading global cause of disability

2.5 Differential Diagnosis of Sadness (Slide 12)

Normal / reactive	Primary mood disorders	Secondary / organic
Grief and bereavement	MDD	Mood disorder due to a **medical illness**
Difficulty coping	Persistent depressive disorder	**Delirium**
Demoralisation	**Bipolar I and II**	**Stroke (CVA)**
Adjustment disorder	**Schizoaffective**	**Substance/medication-induced**

2.6 Major Depressive Episode: SIG-E-CAPS (Slides 13–14) ⭐⭐

Criteria:

- ⭐ **≥ 2 weeks** of **depressed mood and/or anhedonia**
- most of the day, nearly every day
- **plus ≥ 4 more (≥ 5 total)**
- causing distress/impairment
- not due to a substance or medical condition

Letter	Symptom
S	**Sleep**: insomnia or hypersomnia
I	**Interest**: anhedonia

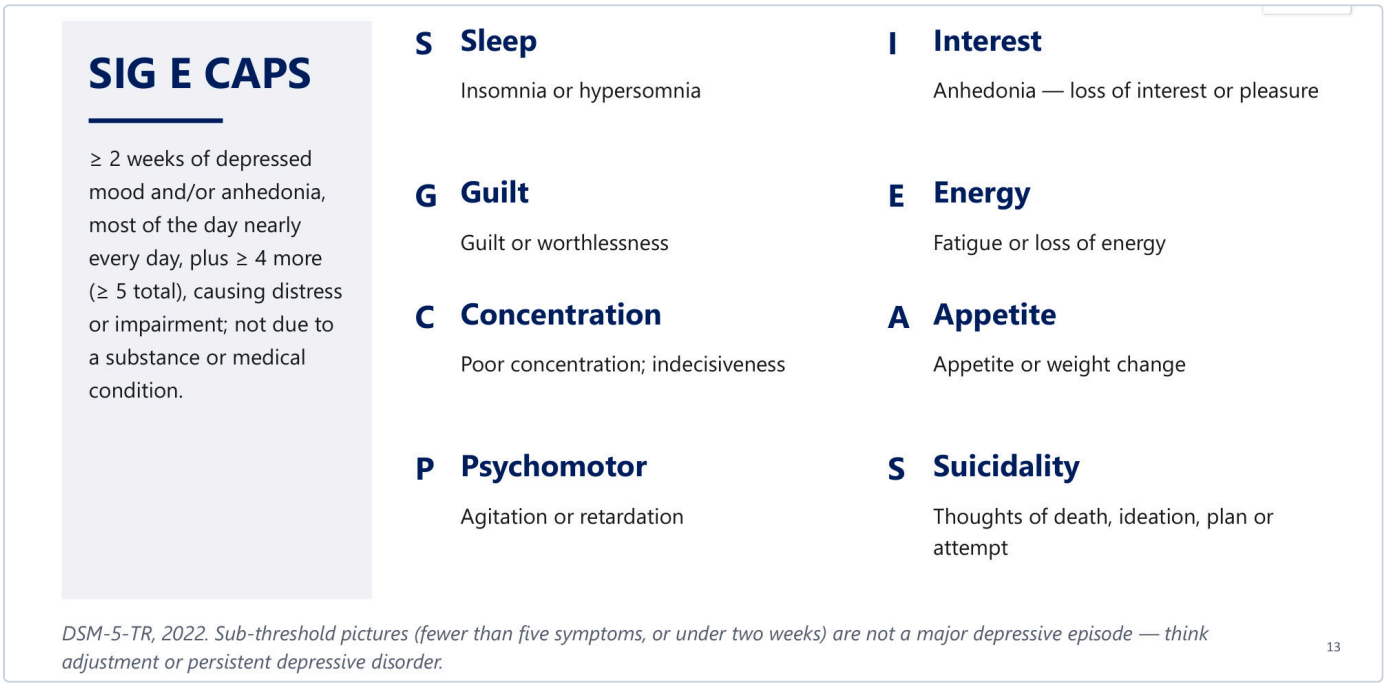
Letter	Symptom
G	**Guilt**/worthlessness
E	**Energy**: fatigue
C	**Concentration**: poor; indecisive
A	**Appetite**/weight change
P	**Psychomotor** agitation or retardation
S	**Suicidality**: thoughts of death, ideation, plan, attempt

Plus the core symptom: depressed mood. *[added]* "SIG: E CAPS" reads like a prescription for "energy capsules". Mood + these 8 items = the 9 DSM criteria.

- Sub-threshold (< 5 symptoms or < 2 weeks) → think **adjustment or persistent depressive disorder**.

MDD (slide 14):

- **unipolar**; single or recurrent episodes; marked distress/impairment
- ⭐ **The bipolar gate: no history of mania or hypomania**. Any such history makes it **bipolar**.
- Psychosis or anxiety may occur (**specifiers**).
- ⭐ Ask directly about **past elevated mood, reduced sleep need, overactivity**. A missed hypomania → misclassified as unipolar → **antidepressant-induced switching**.



Slide 13: SIG-E-CAPS

2.7 Across the Lifespan (Slides 15–16)

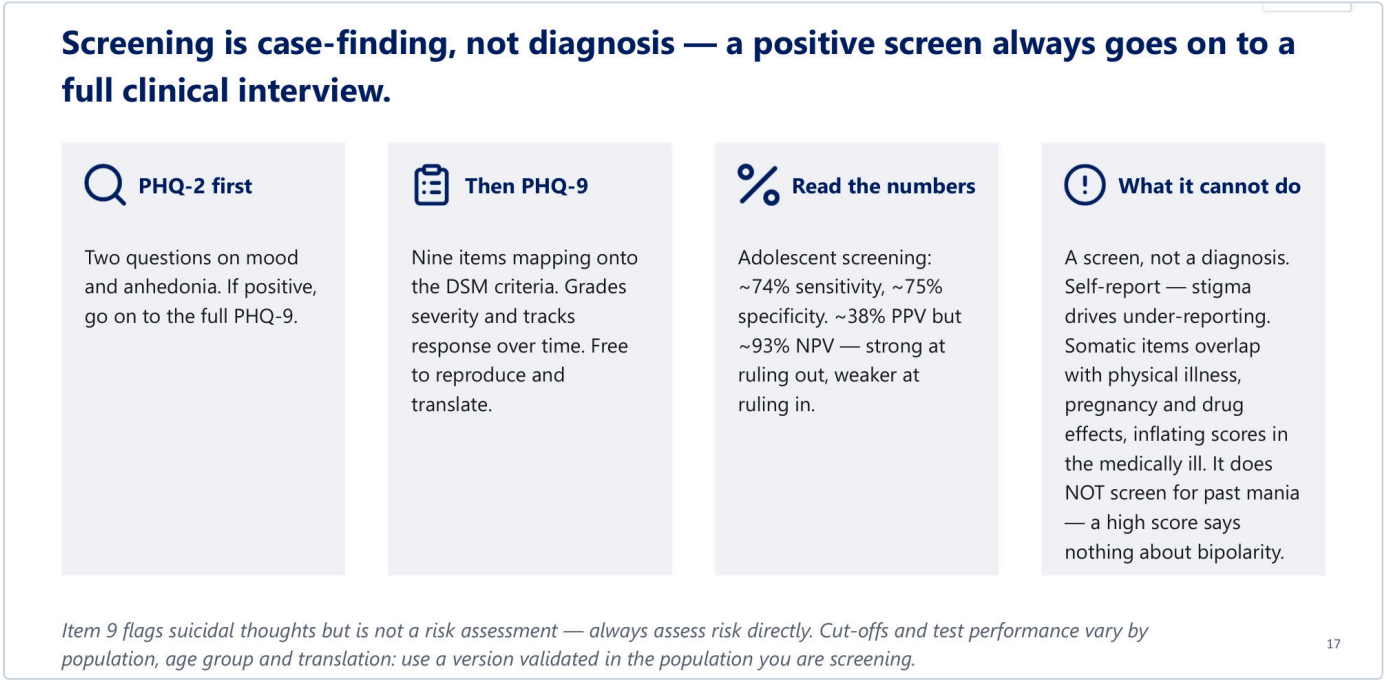
Children/adolescents	Elderly
⭐ **Irritability may replace sadness**	**Not a normal part of ageing**
Somatic complaints; declining school performance	Patients **minimise sadness**; present with **agitation**
Withdrawal, boredom, **school refusal**	⭐ Cognitive slowing mimics dementia (**"pseudodementia"**)

Children/adolescents	Elderly
⭐ **DMDD:** chronic severe irritability + frequent temper outbursts, angry mood between; **onset < 10**; diagnosed **6–18**	⭐ **Highest completed-suicide rates; more lethal methods**
DMDD was created to **reduce over-diagnosis of paediatric bipolar** (chronic irritability ≠ episodic mania)	Under-recognised behind somatic complaints; **screen even when mood is denied**

2.8 Screening and Rating Scales (Slides 17–18)

- **PHQ-2 → PHQ-9.**
  - PHQ-2: 2 items (mood, anhedonia); if positive → PHQ-9.
  - PHQ-9: 9 items mapping onto DSM criteria; grades severity and tracks response; free.
- **Adolescent screening performance:**
  - sensitivity **~74%**, specificity **~75%**
  - **PPV ~38%, NPV ~93%** → ⭐ **good at ruling OUT, weak at ruling in**
- **Limits:**
  - a screen, not a diagnosis
  - self-report (stigma → under-reporting)
  - **somatic items inflate scores in the medically ill**
  - ⭐ **does NOT screen for past mania**
  - **item 9** flags suicidal thoughts but is **not a risk assessment**
- Use a version **validated for the population/language**.

Scale	Type	Best use
PHQ-9	Self-report (9)	Screening, severity, monitoring: the everyday tool
HAM-D	Clinician	Classic research severity
MADRS	Clinician	⭐ **Most sensitive to change** (antidepressant trials)
BDI	Self-report	Severity
⭐ **EPDS (Edinburgh)**	Self-report	**Peripartum** screening
⭐ **YMRS (Young)**	Clinician	**Mania** severity



Slide 17: Screening for depression

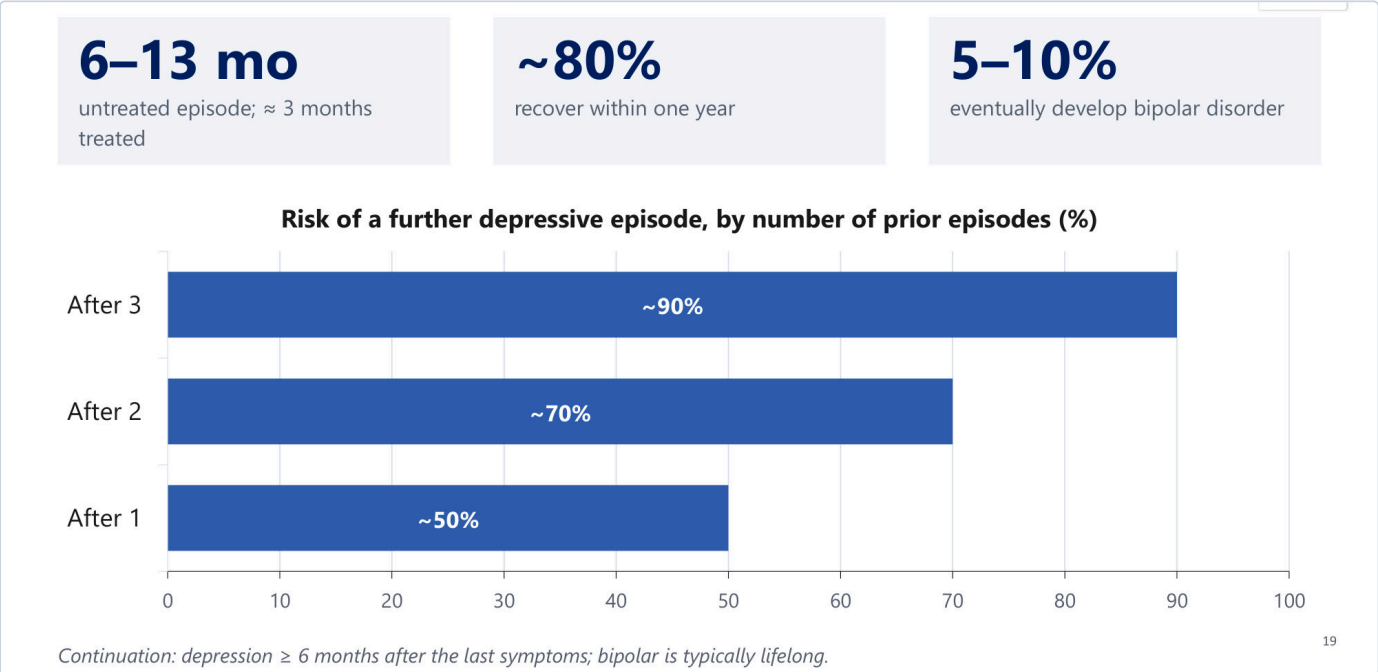
2.9 Course and Prognosis of Depression (Slides 19–21) ⭐

Figure	Value
Episode length	⭐ **6–13 months untreated; ≈ 3 months treated**
Recovery	**~80% within 1 year**
Later bipolar	⭐ **5–10%** eventually develop bipolar disorder
Recurrence after **1** episode	⭐ **~50%**
After **2** episodes	⭐ **~70%**
After **3** episodes	⭐ **~90%**

Systemic illness (slide 20):

- **independent CVD risk factor**; ↑ **T2DM** risk
- raised rates of COPD, asthma, migraine, stroke, epilepsy, MS, thyroid disease, cancer, chronic back pain
- **bidirectional**: treat both

Lower recovery rates	Higher recurrence risk
Long current episode	**Multiple prior episodes**
Greater severity	Previous **severe** episode
Psychotic features	⭐ **Residual symptoms during remission** (a red flag to keep treating)
Prominent **anxiety**	**Younger age of onset**
Comorbid **personality disorder**	—



Slide 19: Recurrence risk by number of episodes

2.10 Suicide in Depression (Slides 22–24) ⭐

- **Best predictor of a future attempt: ⭐ a previous suicide attempt** (lecture MCQ answer B). It outweighs demographics.
- **About two-thirds** of depressed patients contemplate suicide.
- Depression is involved in **40–60%** of completed suicides.
- ⭐ **Low CSF 5-HIAA** (serotonin metabolite) is associated with, and **predicts**, suicidal behaviour.

- **Action:** assess ideation, intent, plan, means, protective factors; **restrict means**; **document a safety plan**. Asking doesn't plant the idea.

2.11 How Well Does Treatment Work? (Slide 25)

- ⭐ Antidepressants roughly **double the placebo response: 50–65% vs 25–30%** over ~3 months.
- **SSRIs block the reuptake pump.** Monoamines rise in hours; ⭐ **onset of effect 4–6 weeks**.
- ⭐ **Continue 6–9 months after remission** (**≥ 2 years if recurrent/high-risk**, slide 43).
- Set expectations: **the drug is working before the patient feels it.**

> 🔁 **Recap: Depression, Part 1.**
>
> - MDD = ≥ 2 weeks, ≥ 5 SIG-E-CAPS (mood or anhedonia required), **no past (hypo)mania**.
> - F:M 2:1. Heritability ~40%. HPA overactivity, ↓ BDNF, short REM latency.
> - Recurrence 50 → 70 → 90%; 5–10% become bipolar.
> - Previous attempt is the strongest suicide predictor; low CSF 5-HIAA.
> - Response 50–65% vs placebo 25–30%; 4–6 weeks to work; continue 6–9 months.
>
> ▶️ **Up next: Secondary causes and the depressive spectrum.**

2.12 Secondary Depression: Medical and Medication Causes (Slides 26–28) ⭐

Case (slides 26–27):

- 48-year-old woman, 3 months of low mood, fatigue, **weight gain**, poor concentration, **constipation**
- **cold intolerance, dry skin, hoarse voice**

→ ⭐ **B: check TSH** (hypothyroidism). Treating the thyroid can resolve mood **without an antidepressant**.

Medical conditions	Medications
⭐ **Hypothyroidism, Cushing's, Addison's**	⭐ **Corticosteroids, anabolic steroids**
⭐ **Pancreatic** and other cancers	⭐ **Interferon**
Post-stroke, post-MI	⭐ **Isotretinoin**
SLE, MS, HIV/AIDS	Some **antihypertensives** and hormonal agents

Screen thyroid function and review the medication list in every new presentation.

2.13 Substance-Induced Mood Disorder (Slides 29–31)

Case (slides 29–30):

- 52-year-old man, 1 month of low mood, poor sleep, anhedonia, irritability
- **drinks heavily most evenings**; **worst in the mornings**; no psychiatric history

→ ⭐ **B: substance-induced mood disorder.**

Common culprits	Approach
Alcohol and sedative-hypnotics (use or withdrawal)	**Temporal link** to use/withdrawal

Common culprits	Approach
Stimulant "crash" (cocaine, amphetamine)	⭐ **Reassess after abstinence**
Cannabis, opioids	**Urine drug screen** supports

Don't commit to primary MDD during active use or acute withdrawal.

2.14 PMDD and Persistent Depressive Disorder (Slides 32–33)

PMDD	Persistent depressive disorder (dysthymia)
⭐ **≥ 5 symptoms in the final week before menses**; improve within days of onset; minimal post-menses	⭐ Depressed mood most days for **≥ 2 years (≥ 1 year in children/adolescents)**
≥ 1 affective symptom: lability, **irritability/anger**, depressed mood/hopelessness, anxiety	**Plus ≥ 2 of 6:** appetite change, sleep disturbance, low energy, **low self-esteem**, poor concentration, **hopelessness**
Others: ↓ interest, poor concentration, fatigue, appetite change, sleep change, overwhelmed, **breast tenderness, bloating**	⭐ **Never symptom-free for > 2 months** in the 2 years
⭐ **Confirmed prospectively over ≥ 2 cycles**	A superimposed MDE = ⭐ **"double depression"**
A formal depressive disorder, not "bad PMS"; key = **late luteal timing**	—

Mnemonic *[added]* for dysthymia: "HE'S 2 SAD":

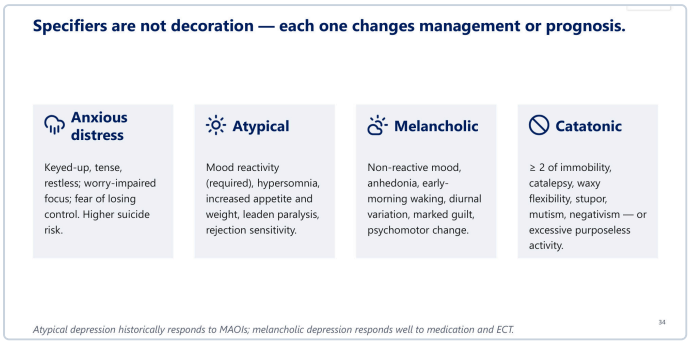
- **H**opelessness
- **E**nergy low
- **S**leep
- **2** years
- **S**elf-esteem low
- **A**ppetite
- **D**ecision/concentration difficulty

2.15 Specifiers (Slides 34–36) ⭐⭐

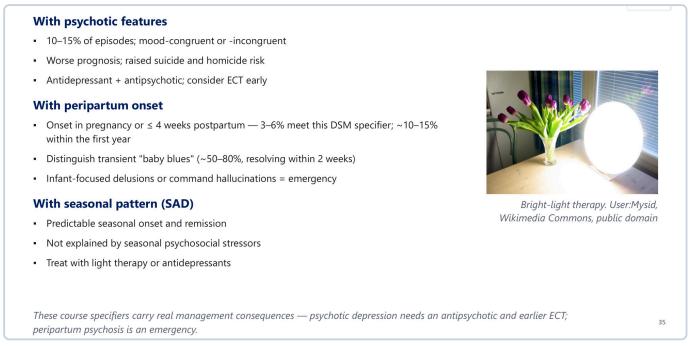
Each specifier changes management or prognosis.

Specifier	Features	Management point
Anxious distress	Keyed-up, tense, restless; worry impairs focus; fear of losing control	⭐ **Higher suicide risk**
⭐ **Atypical**	**Mood reactivity (required)**, **hypersomnia**, **↑ appetite/weight**, **leaden paralysis**, **rejection sensitivity**	Historically responds to **MAOIs**
⭐ **Melancholic**	**Non-reactive mood**, anhedonia, **early-morning waking**, **diurnal variation** (*[added]* worse in mornings), marked guilt, psychomotor change	Responds well to **medication and ECT**
Catatonic	Slide: "≥ 2 of" immobility, catalepsy, waxy flexibility, stupor, mutism, negativism, or excessive purposeless activity	⚠️ **Slide inconsistency:** DSM-5-TR catatonia requires **≥ 3 of 12** features. Lorazepam/ECT (Psychosis lecture)
⭐ **With psychotic features**	**10–15%** of episodes; mood-congruent or incongruent	**Worse prognosis**; ↑ **suicide and homicide** risk. **Antidepressant + antipsychotic; consider ECT early**

Specifier	Features	Management point
⭐ **Peripartum onset**	Onset in **pregnancy or ≤ 4 weeks postpartum**. **3–6%** meet the DSM specifier; **~10–15%** within the first year	Distinguish **baby blues**. **Infant-focused delusions or command hallucinations = emergency**
Seasonal pattern (SAD)	Predictable seasonal onset and remission, not explained by seasonal stressors	⭐ **Light therapy (10,000 lux, morning)** or antidepressants



Slide 34: Symptom specifiers



Slide 35: Course specifiers

The Postpartum Spectrum (Slide 36) ⭐⭐

Feature	Baby blues	Postpartum depression	Postpartum psychosis
Prevalence	⭐ **~50–80%**	⭐ **~10–15%**	⭐ **~1–2 per 1000**
Onset	**Days 3–10**	Within weeks (**up to 12 months**)	**Rapid: days to 2 weeks**
Features	Mild tearfulness, lability, anxiety	Full depressive episode ± anxiety	**Delusions, hallucinations, confusion**, mood instability
Course	⭐ **Self-limiting (≤ 2 weeks)**	Persists; needs treatment	**Fluctuating; deteriorates fast**
Action	Reassurance, support	Psychotherapy ± antidepressant; **EPDS**	⭐ **EMERGENCY ADMISSION**; never manage at home

[added] Postpartum psychosis is strongly linked to **bipolar disorder**.

Feature	Baby blues	Postpartum depression	Postpartum psychosis
Prevalence	~50–80%	~10–15%	~1–2 per 1000
Onset	Days 3–10	Within weeks (up to 12 months)	Rapid — days to 2 weeks
Features	Mild tearfulness, lability, anxiety	Full depressive episode ± anxiety	Delusions, hallucinations, confusion, mood instability
Course	Self-limiting (≤ 2 weeks)	Persists; needs treatment	Fluctuating; deteriorates fast
Action	Reassurance, support	Psychotherapy ± antidepressant; screen (EPDS)	Emergency admission

Postpartum psychosis is a psychiatric emergency — infant-focused delusions or command hallucinations carry real risk to mother and infant. Admit urgently, never manage at home.

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Slide 36: Postpartum spectrum

2.16 Grief vs MDD (Slides 37–39) ⭐

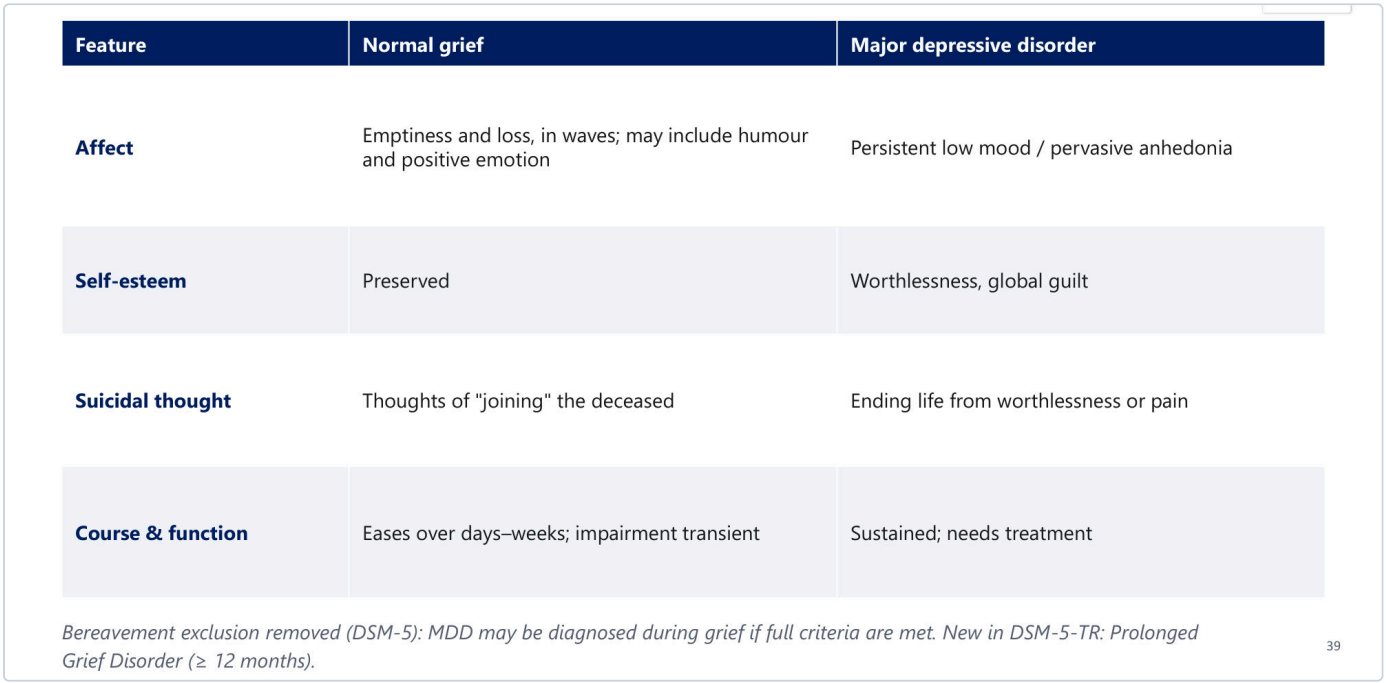
Case (slides 37–38):

- 60-year-old, **3 weeks** after his wife's death
- sadness in **waves**; enjoys his grandchildren
- wishes he "could be with her", but **no wish to end his life**

Which feature would point to MDD? → ⭐ C: pervasive worthlessness and guilt.

Feature	Normal grief	MDD
Affect	Emptiness and loss **in waves**; may include humour, positive emotion	**Persistent** low mood, pervasive anhedonia
Self-esteem	⭐ **Preserved**	⭐ **Worthlessness, global guilt**
Suicidal thought	Thoughts of **"joining" the deceased**	**Ending life** from worthlessness or pain
Course/function	Eases over days–weeks; transient impairment	Sustained; needs treatment

- ⭐ **DSM-5 removed the bereavement exclusion:** MDD **can** be diagnosed during grief if full criteria are met.
- ⭐ **New in DSM-5-TR: Prolonged Grief Disorder (≥ 12 months).**



Feature	Normal grief	Major depressive disorder
Affect	Emptiness and loss, in waves; may include humour and positive emotion	Persistent low mood / pervasive anhedonia
Self-esteem	Preserved	Worthlessness, global guilt
Suicidal thought	Thoughts of "joining" the deceased	Ending life from worthlessness or pain
Course & function	Eases over days–weeks; impairment transient	Sustained; needs treatment

Bereavement exclusion removed (DSM-5): MDD may be diagnosed during grief if full criteria are met. New in DSM-5-TR: Prolonged Grief Disorder (≥ 12 months).

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Slide 39: Grief vs MDD

2.17 Adjustment Disorder vs MDD (Slide 40)

	Adjustment disorder	MDD
Trigger	⭐ **Identifiable stressor within 3 months**	May or may not follow a stressor
Criteria	Distress out of proportion, but **doesn't meet full MDD criteria**	**Meets ≥ 5 symptoms for 2 weeks**
Duration	⭐ **Resolves within 6 months** of the stressor ending	Persists; recurrent
Management	Supportive, psychotherapy, problem-solving	Antidepressant ± psychotherapy

⭐ **If full MDD criteria are met, diagnose MDD**, even with a clear stressor.

2.18 Schizoaffective Disorder (Slide 41)

Criteria:

- **A.** uninterrupted illness with a major mood episode concurrent with schizophrenia Criterion A
- **B.** ⭐ **delusions/hallucinations ≥ 2 weeks without a major mood episode** (lifetime)
- **C.** mood symptoms for the **majority** of the illness
- **D.** not substance/medical

Discriminator:

- **mood disorder with psychosis:** psychosis **only during** mood episodes
- **schizoaffective:** ≥ 2 weeks of psychosis without mood
- **schizophrenia:** mood symptoms brief

2.19 Investigations and Comorbidities (Slide 42)

Routine	Per case	Comorbidities
CBC, TSH, LFTs, electrolytes	⭐ **ECG, especially before TCAs**	Substance use, anxiety, eating disorders
B12, folate	CXR	**Borderline PD**; ADHD, learning and conduct disorders
Urinalysis + urine drug screen	Neurology consult/imaging	⭐ **Men → substance use; women → anxiety/eating**

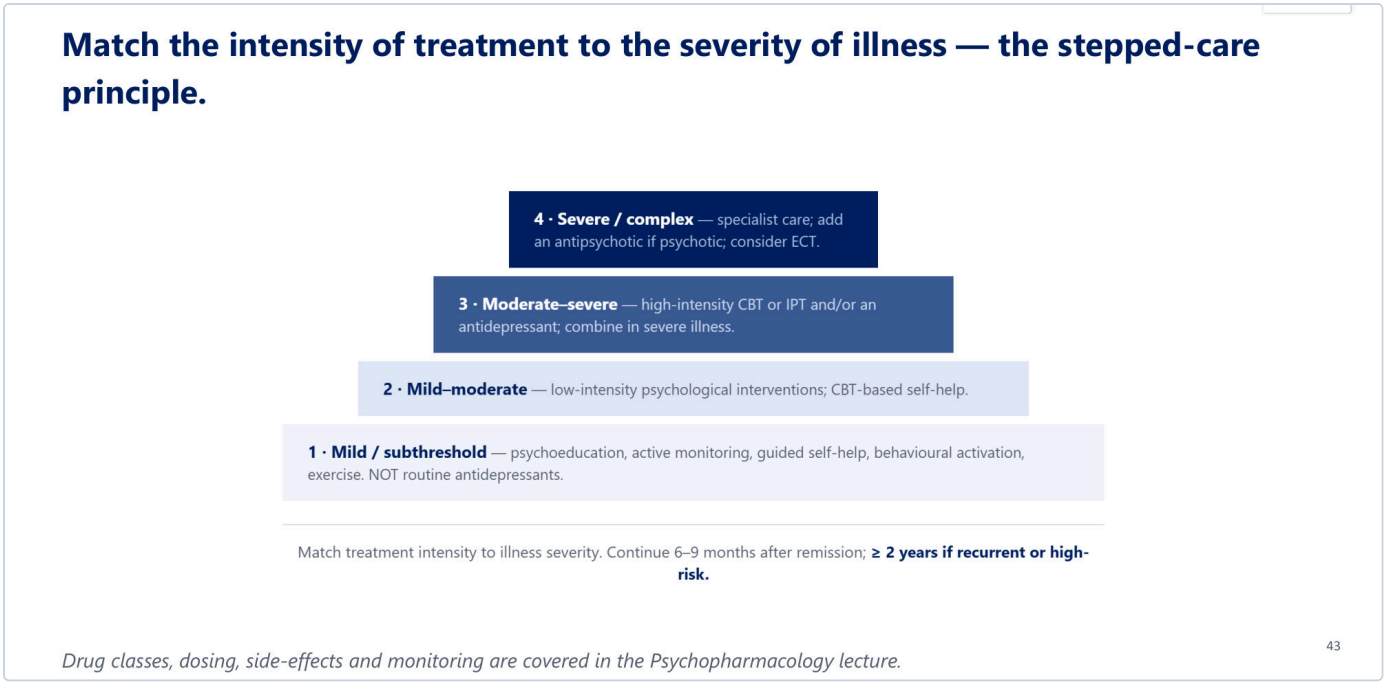
Comorbidity is the rule.

2.20 Management of Depression (Slides 43–45) ⭐⭐

Stepped care (slide 43):

Step	Severity	Treatment
1	**Mild / subthreshold**	Psychoeducation, **active monitoring**, guided self-help, behavioural activation, exercise. ⭐ **NOT routine antidepressants**
2	**Mild–moderate**	**Low-intensity** psychological interventions; CBT-based self-help
3	**Moderate–severe**	**High-intensity CBT or IPT and/or an antidepressant**; ⭐ **combine in severe illness**
4	**Severe / complex**	**Specialist care**; ⭐ **add an antipsychotic if psychotic; consider ECT**

Continue 6–9 months after remission; ≥ 2 years if recurrent or high-risk.



Slide 43: Stepped care

Psychotherapies (slide 44): first-line for mild–moderate; **combined with medication, superior to either alone** in severe illness.

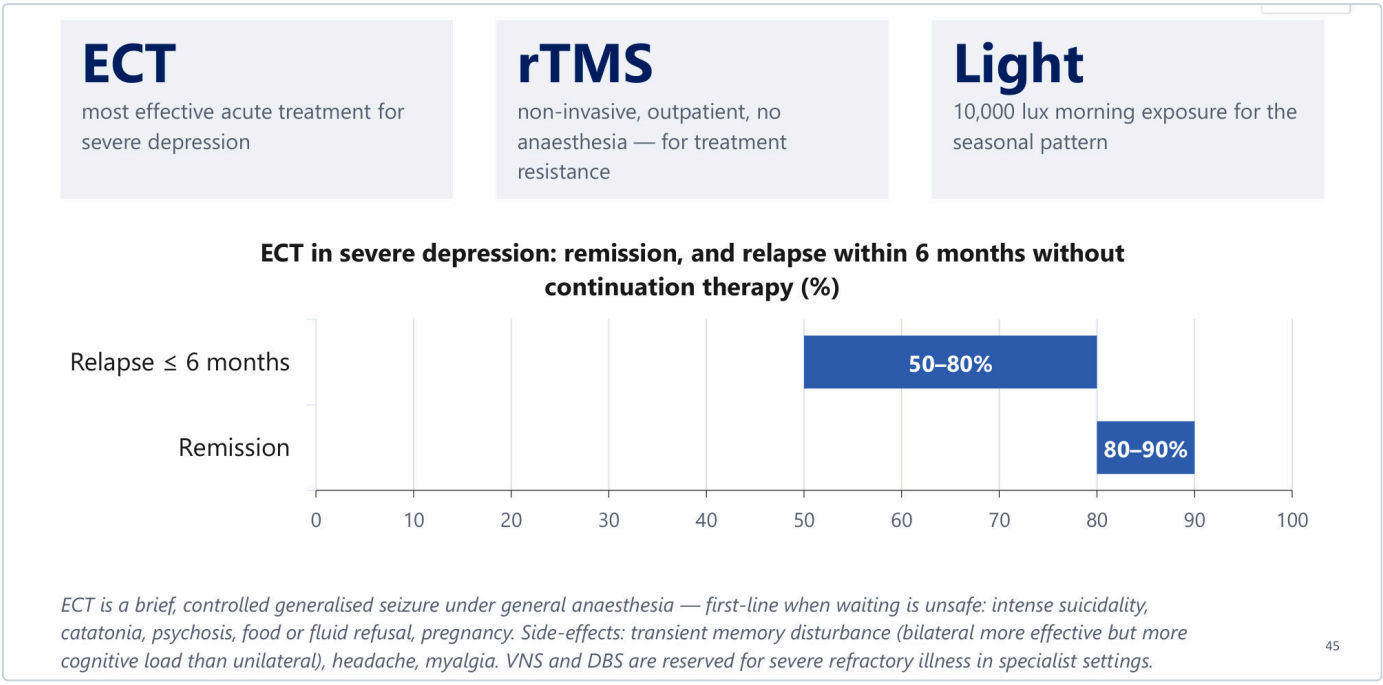
Therapy	Content
⭐ **CBT**	Restructures negative automatic thoughts and the **cognitive triad**; behavioural experiments. **Strongest evidence; prevents relapse**
⭐ **IPT**	Time-limited; current relationships: **grief, role transitions, role disputes, interpersonal deficits**
Behavioural activation	Re-engages rewarding activity to break the withdrawal–low-mood cycle
MBCT	Mindfulness + cognitive skills; ⭐ **reduces relapse in recurrent depression**
Family-focused therapy (bipolar)	Psychoeducation + communication; **lowers expressed emotion** and relapse
⭐ **IPSRT** (bipolar)	**Stabilises sleep–wake and social rhythms**

Neurostimulation (Slide 45)

Modality	Use
⭐ **ECT**	**Most effective acute treatment for severe depression.** Remission **80–90%**; relapse **50–80% within 6 months without continuation therapy**
rTMS	Non-invasive, outpatient, **no anaesthesia**; for **treatment resistance**
Light	**10,000 lux morning** for the seasonal pattern
VNS, DBS	Severe refractory illness, specialist settings

- **ECT** is a brief, controlled **generalised seizure under general anaesthesia**.
- ⭐ **First-line when waiting is unsafe:**
  1. **intense suicidality**
  2. **catatonia**
  3. **psychosis**
  4. **food/fluid refusal**
  5. **pregnancy**

- **Side-effects:** transient **memory disturbance** (**bilateral more effective, more cognitive load** than unilateral), headache, myalgia.



Slide 45: ECT, rTMS, light

🔁 **Recap: Depression, Part 2.**

- Exclude hypothyroidism, Cushing/Addison, cancer (pancreas), steroids, interferon, isotretinoin, alcohol.
- PMDD (luteal, ≥ 2 cycles). Dysthymia (≥ 2 years, never > 2 months well; double depression).
- Specifiers: atypical (MAOIs), melancholic (meds/ECT), psychotic (+ antipsychotic, ECT), peripartum, seasonal (light).
- Blues / PPD / psychosis = 50–80% / 10–15% / 1–2 per 1000.
- Grief preserves self-esteem; prolonged grief ≥ 12 months. Adjustment: < 3 months after a stressor, resolves within 6 months.
- Stepped care 1–4; ECT for unsafe-to-wait cases.

▶️ **Up next: The other pole: mania and bipolar disorder.**

2.21 Bipolar: Opening Case Mrs. R, 31 (Slide 48)

Presentation:

- presents with **depression** (low mood, poor appetite, concentration, worthlessness)
- **2 years ago, 4 days before her sister's wedding:**
  - slept **2 h/night**, "wasn't tired"
  - "**brilliant ideas**" racing
  - worked all night making pastries/gifts; **kept working productively** at her job
  - inappropriate with men at the wedding
- no substances

→ ⭐ **Bipolar II**: a discrete **≥ 4-day hypomanic episode** (no marked impairment, no hospitalisation, no psychosis) + current major depression.

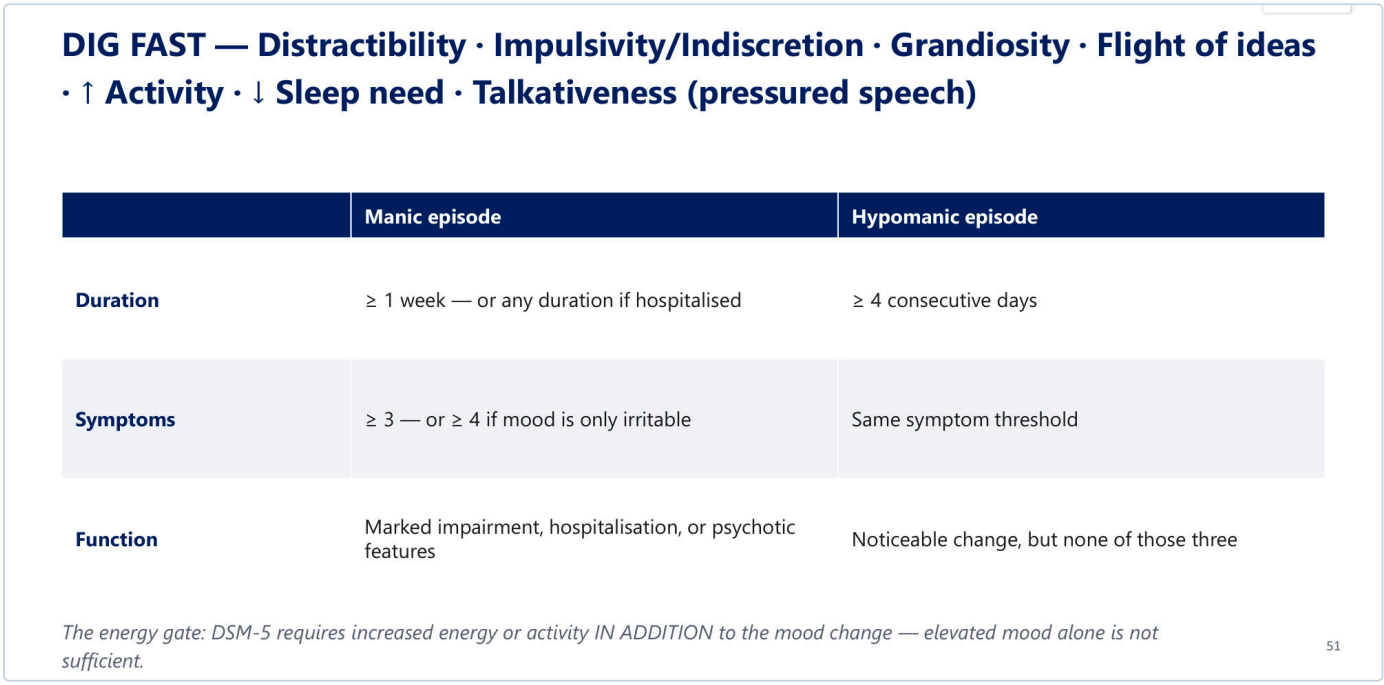
2.22 Mania vs Hypomania (Slides 49–51) ⭐⭐

DIG FAST:

- **D**istractibility
- **I**mpulsivity/Indiscretion
- **G**randiosity
- **F**light of ideas
- ↑ **A**ctivity
- ↓ **S**leep need
- **T**alkativeness (pressured speech)

	Manic episode	Hypomanic episode
Duration	⭐ **≥ 1 week**, or **any duration if hospitalised**	⭐ **≥ 4 consecutive days**
Symptoms	⭐ **≥ 3**, or **≥ 4 if mood only irritable**	Same threshold
Function	⭐ **Marked impairment, hospitalisation, or psychotic features**	Noticeable change, but **none of those three**

- ⭐ **The energy gate:** DSM-5 requires **increased energy/activity IN ADDITION to the mood change**; elevated mood alone is not sufficient.
- **Case (slides 49–50):** 26-year-old, **5 days** elevated mood, ↓ sleep need, rapid speech, ↑ activity, **functioning well at work**, no psychosis, no admission → ⭐ **B: hypomanic episode**. Add any one of impairment, hospitalisation or psychosis and it becomes mania.

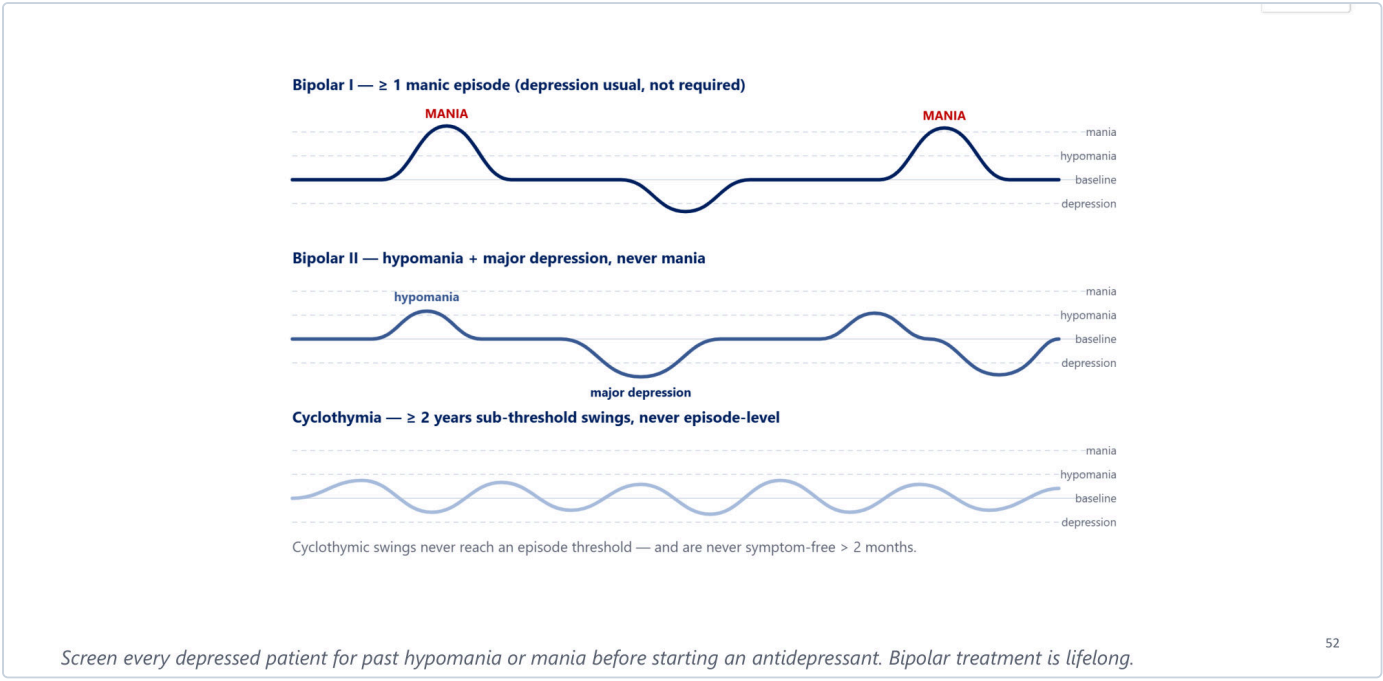


Slide 51: Mania vs hypomania, DIG FAST

2.23 Bipolar I vs II vs Cyclothymia (Slide 52) ⭐

Disorder	Requirement
⭐ **Bipolar I**	**≥ 1 manic episode**; depression usual but **not required**
⭐ **Bipolar II**	**Hypomania + major depression; never mania**
⭐ **Cyclothymia**	**≥ 2 years** of **sub-threshold** swings, **never episode-level**; never symptom-free > 2 months

⭐ Screen every depressed patient for past hypomania/mania before starting an antidepressant. Bipolar treatment is lifelong.



Slide 52: Bipolar I vs II vs cyclothymia

2.24 Specifiers: Rapid Cycling and Mixed Features (Slides 53–54) ⭐

Bipolar specifiers: rapid cycling · psychotic · melancholic · atypical · catatonic · peripartum · seasonal.

Rapid cycling	Mixed features
⭐ **≥ 4 mood episodes in 12 months**, separated by ≥ 2 months' remission or a switch to the opposite pole	A mood episode with ⭐ **≥ 3 symptoms of the opposite pole at the same time**
Commoner in **bipolar II** and **women**	Depressed but **agitated, racing thoughts, irritable**; or manic but **tearful, hopeless, guilty**
Earlier onset, longer illness	Mistaken for agitated depression or anxiety
⭐ **More suicide attempts** and morbidity	⭐ **HIGHEST suicide risk of any mood presentation**
A **course specifier**, not a separate disorder	⭐ **Antidepressant monotherapy can worsen it**: use a **mood stabiliser/atypical**

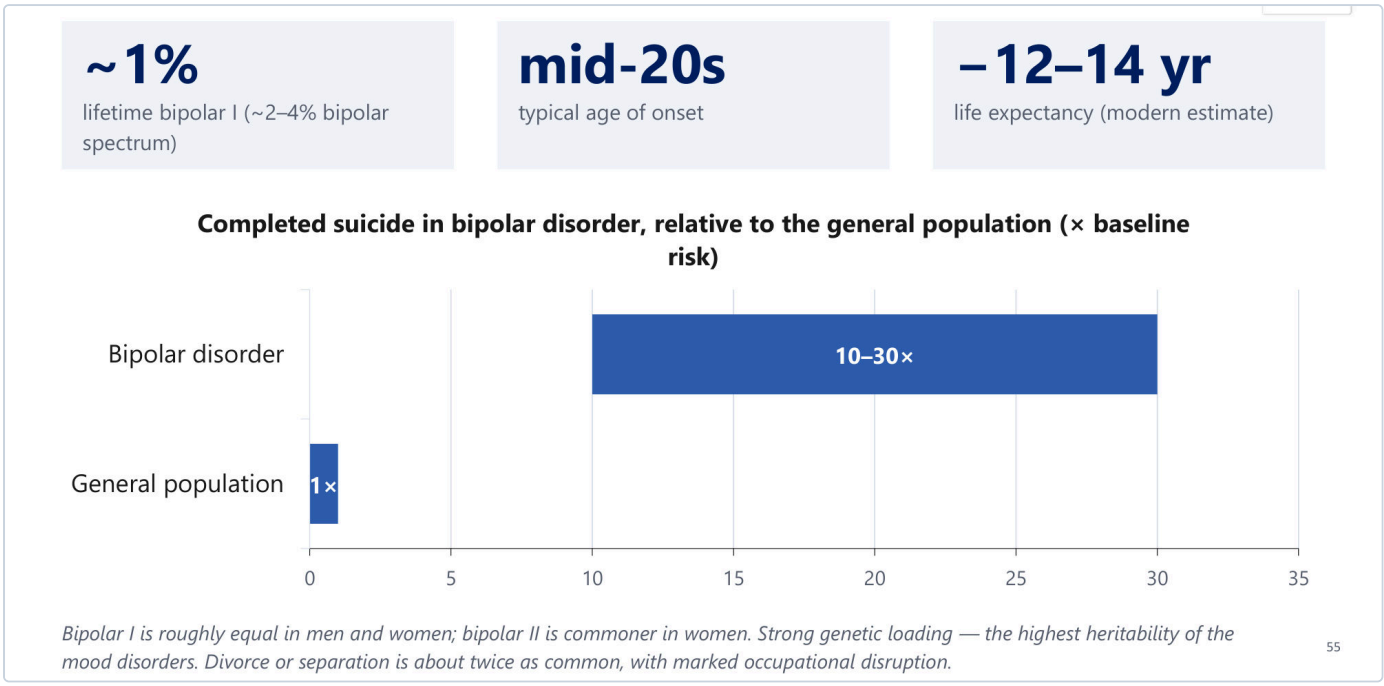
2.25 Burden, Prognosis and Suicide in Bipolar (Slides 55–56)

Figure	Value
Lifetime bipolar I	⭐ **~1%** (**2–4%** bipolar spectrum)
Typical onset	**Mid-20s**
Life expectancy	⭐ **−12–14 years**
Completed suicide	⭐ **≈ 6–7%** (older inpatient estimates **15–20%**); ⭐ **10–30× the general population**
Sex	Bipolar I ≈ **M = F**; bipolar II **commoner in women**
Genetics	⭐ **Highest heritability of the mood disorders**
Social	Divorce/separation ~**2×**; marked occupational disruption

- **Poor prognosis:**

- psychotic features; mixed episodes
- **rapid cycling**
- comorbid substance use
- poor adherence
- early onset; frequent episodes

- ⭐ **Suicide risk: bipolar II particularly at risk**; highest in **depressive and mixed** states.



Slide 55: Bipolar burden

2.26 Differential Diagnosis of Bipolar (Slides 57–59)

- **Consider:**
  - unipolar MDD
  - **cocaine/amphetamine**
  - **borderline/narcissistic PD**
  - organic/secondary
  - schizoaffective
- **Unifying test: ⭐ episodicity with a clear return to baseline**, corroborated by **collateral**.

Contrast	Points to the mimic	Points to bipolar
Substance use	Symptoms tied to intoxication/withdrawal; ⭐ **6-12 months' abstinence to judge**	Bipolar usually **precedes** substance use (and is a risk factor for it)
ADHD	⭐ **Onset < 12, continuous**, responds to stimulants; FHx of disruptive disorders	**Later onset, episodic**, FHx of mood disorder, responds to mood stabilisers
Borderline PD	Dysfunction **persists between episodes**; mood often sub-threshold; history of deprivation/abuse	**Biphasic episodes meeting threshold**; reasonable function when euthymic; FHx of bipolar

Secondary mania (slide 59):

Medical	Drugs/substances
⭐ **Hyperthyroidism, Cushing's**	⭐ **Corticosteroids**
MS, stroke	⭐ **Antidepressants (switch)**
Temporal/frontal lobe pathology	**Stimulants, cocaine, amphetamine**

Medical	Drugs/substances
Neurosyphilis, HIV	**L-dopa**, sympathomimetics

A first manic episode at an atypical age or with focal neurological signs → organic work-up first.

Contrast	Points toward the mimic	Points toward bipolar
Substance use	Symptoms tied to intoxication or withdrawal; needs 6–12 months' abstinence to judge	Bipolar usually precedes the substance use (and is a risk factor for it)
ADHD	Onset < 12, continuous, responds to stimulants; family history of disruptive disorders	Onset later, episodic, family history of mood disorder, responds to mood stabilisers
Borderline PD	Dysfunction persists between episodes; mood often sub-threshold; history of deprivation or abuse	Biphasic episodes meeting threshold; reasonable function when euthymic; family history of bipolar

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Slide 58: Bipolar vs substance, ADHD, BPD

2.27 Management of Bipolar (Slides 60–63) ⭐⭐

Case (slides 60–61): a known bipolar I patient in a depressive episode; a colleague suggests SSRI monotherapy. Main risk → ⭐ **B: switch into mania or rapid cycling.**

Phase	Approach	Key rule
⭐ **Acute mania**	**Mood stabiliser (lithium or valproate) and/or antipsychotic**; short-term **benzodiazepine** for agitation/sleep	⭐ **STOP antidepressants**
⭐ **Bipolar depression**	**Quetiapine, lurasidone, cariprazine; lamotrigine; or lithium**	⭐ **Avoid antidepressant monotherapy**; if used at all, **on top of a mood stabiliser**
⭐ **Maintenance (lifelong)**	⭐ **Lithium first-line: gold standard and ANTI-SUICIDE.** Alternatives: valproate, lamotrigine, atypicals	Treat to full remission; **relapse is the rule if stopped**
Severe/refractory/mixed/pregnancy	⭐ **ECT**	Most effective for **severe and mixed** states

Psychosocial care (slide 63):

- **psychoeducation** with a **written relapse-prevention plan** (early-warning signs)
- **family-focused therapy** (↓ expressed emotion)
- ⭐ **IPSRT** (regular sleep–wake routines; **rhythm disruption precipitates episodes**)
- lifestyle and adherence: sleep hygiene, avoid substances, monitoring

[added] Valproate is avoided in women of childbearing potential (teratogenic); lithium needs level, renal and thyroid monitoring (covered in the Psychotropics lecture).

Phase	Approach (decision level)	Key rule
Acute mania	Mood stabiliser (lithium or valproate) and/or an antipsychotic; short-term benzodiazepine for agitation and sleep	Stop antidepressants
Bipolar depression	Quetiapine, lurasidone, or cariprazine; lamotrigine; or lithium	Avoid antidepressant monotherapy — switch and cycling risk
Maintenance (lifelong)	Lithium first-line — gold standard and anti-suicide. Alternatives: valproate, lamotrigine, atypicals	Treat to full remission; relapse is the rule if stopped
Severe / refractory / mixed / pregnancy	ECT	Most effective for severe and mixed states

Agent selection at decision level only — doses, blood levels, side-effects and monitoring are covered in the Psychopharmacology lecture.

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Slide 62: Bipolar management by phase

> 🔁 **Recap: Bipolar.**
> - DIG FAST + the **energy gate**.
> - Mania ≥ 1 week (or hospitalised) with impairment, hospitalisation or psychosis. Hypomania ≥ 4 days, none of these.
> - Bipolar I = mania. Bipolar II = hypomania + MDE. Cyclothymia = ≥ 2 years sub-threshold.
> - Rapid cycling ≥ 4 episodes/year. Mixed features = highest suicide risk.
> - ~1%, mid-20s, −12–14 years, suicide 6–7% (10–30×).
> - Mania: lithium/valproate ± antipsychotic, stop antidepressants. Depression: quetiapine, lurasidone, cariprazine, lamotrigine, lithium. Maintenance: **lithium** (anti-suicide). ECT for severe/mixed/pregnancy.

2.28 Integrative Case: Ms. F, 26 (Slides 65–66) ⭐

Presentation:

- teacher, **3rd depressive episode since age 19**
- for 2 months: hypersomnia, ↑ appetite/weight gain, **leaden limbs**, withdrawn after "the smallest criticism"
- GP started **fluoxetine 4 weeks ago**
- last **6 days**: sleeps 3 h and not tired, rapid talk, reorganising the flat at 3 a.m., spent most of her salary, "**better than well**"
- no hospitalisation, no psychosis, **still teaching** (head teacher comments)
- **maternal uncle on lithium** for "manic depression"

Question	Model answer (lecture)
1. Depressive specifier?	⭐ **Atypical features**: hypersomnia, hyperphagia/weight gain, leaden paralysis, rejection sensitivity

Question	Model answer (lecture)
2. Mania or hypomania?	⭐ **Hypomania**: ≥ 4 days, **no** marked impairment, psychosis or hospitalisation
3. Diagnosis?	⭐ **Bipolar II**. The earlier "depressive" episodes were the **depressive pole**
4. What went wrong?	**Antidepressant monotherapy in undiagnosed bipolar.** Missed clue: **family history + recurrent early-onset depression**
5. Management?	**Stop/taper the antidepressant; start a mood stabiliser**; then maintenance, psychoeducation, **rhythm stabilisation**, suicide-risk monitoring

⭐ **Hypomania is rarely volunteered because it feels good**: ask directly, with a collateral informant.

[added] "Bipolar red flags" in a depressed patient:

- **early onset**
- **recurrent** episodes
- **atypical features**
- **psychotic or postpartum** depression
- **family history of bipolar**
- **antidepressant-induced activation**

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3. High-Yield Comparison Tables

Table A: Depressive Disorders

Disorder	Duration	Core
MDD	≥ 2 weeks	≥ 5 SIG-E-CAPS incl. mood/anhedonia; no (hypo)mania ever
Persistent depressive	≥ 2 years (≥ 1 year in children)	Low mood + ≥ 2 of 6; never well > 2 months
PMDD	Late luteal phase; ≥ 2 cycles prospectively	≥ 5 symptoms, ≥ 1 affective
DMDD	Onset < 10; diagnosed 6–18	Chronic irritability + outbursts
Adjustment	Within 3 months of a stressor; resolves by 6 months	Sub-threshold
Prolonged grief	≥ 12 months	DSM-5-TR addition

Table B: Mood Episodes

	Mania	Hypomania	MDE
Duration	≥ 7 days (any if admitted)	≥ 4 days	≥ 2 weeks
Symptoms	≥ 3 DIG FAST (≥ 4 if irritable)	Same	≥ 5 SIG-E-CAPS
Impairment/psychosis/admission	**Yes (any one)**	**None**	Distress/impairment

Table C: Bipolar I vs II vs Cyclothymia vs MDD

	Bipolar I	Bipolar II	Cyclothymia	MDD
Mania	✅	❌	❌	❌

	Bipolar I	Bipolar II	Cyclothymia	MDD
Hypomania	±	✅	Sub-threshold	❌
MDE	Usual, not required	✅ Required	Sub-threshold	✅
Sex	M = F	F > M	—	F 2:1

Table D: Specifier → Management

Specifier	Management consequence
Atypical	MAOIs historically
Melancholic	Medication, ECT
Psychotic	Antidepressant + antipsychotic; early ECT
Anxious distress	↑ suicide risk
Catatonic	Lorazepam, ECT
Peripartum psychosis	Emergency admission
Seasonal	Light therapy 10,000 lux
Mixed features	Avoid antidepressant monotherapy; mood stabiliser/atypical; highest suicide risk
Rapid cycling	Harder to treat; commoner in BP-II/women

Table E: Bipolar vs Its Mimics

Mimic	Key discriminator
BPD	Hours-long, interpersonally triggered; dysfunction persists between
ADHD	Onset < 12, continuous, stimulant-responsive
Substance	Linked to use/withdrawal; reassess after 6–12 months abstinent
Unipolar MDD	No (hypo)mania ever; ask directly + collateral
Secondary mania	Atypical age, neuro signs, thyroid/steroids

Table F: Key Numbers

Item	Number
Depression worldwide	~280 M; #1 cause of disability
Lifetime MDD	M 5–12%, F 10–25%
Heritability (MDD)	~40%
Depressed patients in completed suicides	40–60%
Antidepressant vs placebo	50–65% vs 25–30%
Recurrence	50% / 70% / 90% after 1/2/3 episodes
MDD → bipolar	5–10%
Psychotic features	10–15% of episodes
Baby blues / PPD / PP psychosis	50–80% / 10–15% / 1–2 per 1000
Bipolar I	~1% (spectrum 2–4%); onset mid-20s
Bipolar suicide	6–7% (older 15–20%); 10–30×
Bipolar life expectancy	−12–14 years
ECT	Remission 80–90%; relapse 50–80% by 6 months without continuation

4. Rules, Exceptions, Patterns

- **No past (hypo)mania** is part of the MDD definition. **Always screen**, with collateral.
- **Energy/activity must rise** along with mood for (hypo)mania.
- **Any hospitalisation or psychosis = mania**, regardless of duration.
- **Bipolar I needs mania only**; depression is not required.
- **Antidepressant monotherapy is avoided in bipolar** (switch, rapid cycling, mixed states).
- **Lithium** = gold-standard maintenance + **anti-suicidal**.
- **Mixed features** = highest suicide risk.
- **Bereavement exclusion removed**: MDD can coexist with grief. Prolonged grief ≥ 12 months.
- **Full MDD criteria override adjustment disorder.**
- **Children: irritability** can replace sadness; DMDD ≠ paediatric bipolar.
- **Elderly:** pseudodementia; highest completed suicide.
- **PHQ-9:** rules out better than it rules in; doesn't detect mania; item 9 ≠ risk assessment.
- **Residual symptoms → keep treating.**
- **Check TSH and the drug list** in every new depression.
- **Atypical (reactive mood) vs melancholic (non-reactive, early waking, worse in the morning).**
- **ECT first-line** when waiting is unsafe: suicidality, catatonia, psychosis, refusal to eat/drink, pregnancy.

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5. Clinical Correlations

- **Endocrine:**
  - **hypothyroidism** (weight gain, cold intolerance, constipation, hoarse voice) mimics depression
  - **Cushing's** causes depression (or mania)
  - **Addison's** causes apathy/depression
  - **hyperthyroidism** can precipitate mania
  - **dexamethasone non-suppression** in depression echoes the Cushing work-up (Adrenal I)
- **Cardiology:** depression is an independent CVD risk factor; **post-MI depression** worsens outcomes; **ECG before TCAs**.
- **Oncology: pancreatic cancer** classically presents with depression.
- **Dermatology: isotretinoin** → depression/suicidality.
- **Hepatology/ID: interferon** → depression.
- **Obstetrics: EPDS** screening; postpartum psychosis = emergency admission.
- **Steroids:** can cause depression **or** mania.
- **Primary care:** PHQ-2 → PHQ-9; stepped care; don't give routine antidepressants for mild depression.

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6. Rapid Revision (Last-Day)

1. **Sadness vs depression:** proportionate/transient vs ≥ 2 weeks, out of proportion, impairing.
2. **Depressive spectrum:** MDD, PDD, PMDD, DMDD, secondary. **Bipolar spectrum:** I, II, cyclothymia, secondary.
3. **Epidemiology:** ~280 M; #1 disability; F:M 2:1; first-degree relatives 2–3×; heritability ~40%.
4. **Neurobiology:**

- monoamines
- **HPA ↑ cortisol**, dexamethasone non-suppression
- ↓ BDNF/hippocampus
- **short REM latency**
- cognitive triad, learned helplessness

5. **MDD:** ≥ 2 weeks, ≥ 5 **SIG-E-CAPS** (mood or anhedonia), **never (hypo)mania**.
6. **Children:** irritability; **DMDD** (onset < 10, diagnosed 6–18). **Elderly:** pseudodementia, highest suicide.
7. **PHQ-9:** NPV 93%, PPV 38%; no mania detection. EPDS = peripartum. YMRS = mania. MADRS = sensitive to change.
8. **Course:** 6–13 months untreated (3 months treated); 80% recover in 1 year; recurrence 50/70/90%; 5–10% → bipolar.
9. **Suicide:** previous attempt is the best predictor; 2/3 contemplate; 40–60% of suicides; **low CSF 5-HIAA**.
10. **Treatment effect:** 50–65% vs 25–30%; 4–6 weeks; continue 6–9 months (≥ 2 years if recurrent).
11. **Mimics:** - **hypothyroidism**, Cushing, Addison - **pancreatic cancer**, post-stroke/MI - SLE, MS, HIV - **steroids, interferon, isotretinoin**, antihypertensives - **alcohol** (reassess after abstinence)
12. **PMDD:** ≥ 5 in the luteal week, ≥ 2 cycles. **PDD:** ≥ 2 years + ≥ 2 of 6; never well > 2 months; double depression.
13. **Specifiers:** - anxious (↑ suicide) - **atypical** (reactive mood, hypersomnia, hyperphagia, leaden paralysis, rejection sensitivity → MAOI) - **melancholic** (non-reactive, early waking, worse in the morning → medication/ECT) - catatonic - **psychotic** (10–15%; + antipsychotic, ECT) - **peripartum** (≤ 4 weeks) - **seasonal** (light)
14. **Postpartum:** blues 50–80%, days 3–10, ≤ 2 weeks · PPD 10–15%, up to 12 months · **psychosis 1–2/1000, days–2 weeks → admit**.
15. **Grief vs MDD:** self-esteem preserved vs **worthlessness/guilt**. Bereavement exclusion removed. **Prolonged grief ≥ 12 months.**
16. **Adjustment:** within 3 months of a stressor, resolves by 6 months; full criteria → MDD.
17. **Stepped care:** 1 mild (no routine antidepressants) → 2 low-intensity → 3 CBT/IPT ± antidepressant → 4 specialist, antipsychotic, ECT.
18. **Therapies:** CBT (best evidence), IPT (grief, roles), BA, **MBCT (relapse)**, FFT and **IPSRT** (bipolar).
19. **ECT:** most effective acute treatment; suicidality, catatonia, psychosis, food refusal, pregnancy; transient memory loss (bilateral > unilateral).
20. **DIG FAST + energy gate.** Mania ≥ 1 week or hospitalised; impairment, psychosis or admission. Hypomania ≥ 4 days, none.
21. **BP-I** = mania · **BP-II** = hypomania + MDE · **cyclothymia** = ≥ 2 years sub-threshold.
22. **Rapid cycling** ≥ 4/year (BP-II, women). **Mixed** ≥ 3 opposite-pole symptoms → **highest suicide risk**.
23. **Bipolar:** ~1%, mid-20s, −12–14 years, suicide 6–7% (10–30×), highest heritability; BP-II commoner in women.
24. **Mimics:** BPD, ADHD (< 12, continuous), substances (6–12 months abstinent), secondary mania (thyroid, steroids, L-dopa, antidepressants).
25. **Bipolar Rx:** - mania: Li/valproate ± antipsychotic, **stop antidepressants** - depression: quetiapine/lurasidone/cariprazine/lamotrigine/Li - maintenance: **Li** (anti-suicide) - ECT - IPSRT, FFT
26. **TCAs** = most dangerous in overdose (QRS widening, arrhythmia).

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7. Exam-Style Questions

Q1 (lecture, slides 26–27). A 48-year-old woman has 3 months of low mood, fatigue, weight gain, poor concentration and constipation, plus cold intolerance, dry skin and a hoarse voice. Most

important next step? A. Start an SSRI · **B. Check thyroid function (TSH)** · C. Refer for CBT · D. Reassure, review in 6 weeks

> **Answer: B** (lecture key). Classic hypothyroidism mimicking depression. Exclude medical and medication causes before diagnosing primary MDD.

Q2 (lecture, slides 37–38). Three weeks after his wife's death, a 60-year-old has sadness in waves, enjoys his grandchildren, wishes he "could be with her" but has no wish to end his life; sleep is poor. Which feature would point to MDD rather than grief? A. Sadness in waves · B. Preserved self-esteem · **C. Pervasive worthlessness and guilt** · D. Wishing to be with the deceased

> **Answer: C** (lecture key).
> - Normal grief preserves self-esteem, comes in waves and allows pleasure.
> - Global worthlessness/guilt and pervasive anhedonia point to MDD, which since DSM-5 **can** be diagnosed during bereavement.

Q3 (lecture, slides 49–50). A 26-year-old has 5 days of elevated, expansive mood, reduced need for sleep, rapid speech and increased activity; he is more productive and functioning well at work. No psychosis, no admission. Which episode? A. Manic · **B. Hypomanic** · C. Normal happiness · D. Cyclothymia

> **Answer: B** (lecture key).
> - ≥ 4 days with mood + energy change, but **no** marked impairment, hospitalisation or psychosis.
> - Any one of those → mania.

Q4 (lecture, slides 60–61). A patient with known bipolar I disorder presents with a major depressive episode. A colleague suggests SSRI monotherapy. Main risk? A. It's correct first-line · **B. Switch into mania or rapid cycling** · C. Serotonin syndrome · D. No particular risk

> **Answer: B** (lecture key).
> - Treat bipolar depression with **quetiapine, lurasidone, cariprazine, lamotrigine or lithium**.
> - If an antidepressant is used at all, it goes on top of a mood stabiliser.

Q5 (lecture, slides 29–30). A 52-year-old man has 1 month of low mood, poor sleep, anhedonia and irritability; he drinks heavily most evenings; symptoms worst in the mornings; no psychiatric history. Most likely? A. MDD · **B. Substance-induced mood disorder** · C. Adjustment disorder · D. Bipolar depression

> **Answer: B** (lecture key).
> - Alcohol commonly produces depressive states that lift with abstinence.
> - Reassess after a substance-free interval, with a UDS.

Lecture board one-liners (slide 67):

#	Question	Answer
1	31-year-old, current depression; 2 years ago 4 days ↓ sleep, ↑ energy, racing ideas, overspending; no admission or psychosis	**Bipolar II**
2	Most dangerous antidepressant class in overdose	**TCAs**: cardiotoxic, QRS widening, arrhythmia

#	Question	Answer
3	Single best feature separating mania from hypomania	**Marked impairment / hospitalisation / psychosis** (≥ 1 week)
4	Mood stabiliser with the best anti-suicide evidence	**Lithium** (monitor levels, renal, thyroid)
5	22-year-old, 4 months low mood after her father's death; fluctuating and improving	**Normal grief**: fluctuating, improving, function preserved

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8. Exam Pitfalls & Traps

Trap	Correct thinking
Starting an SSRI without asking about past highs	❌ **Screen for (hypo)mania with collateral** in every depressed patient
Elevated mood alone = hypomania	❌ Needs ↑ **energy/activity** too (energy gate)
3 days of mania with admission ≠ mania (too short)	❌ **Any duration if hospitalised** = mania
Hypomania with psychosis	❌ Psychosis makes it **mania** by definition
Bipolar I requires a depressive episode	❌ **One manic episode is enough**
SSRI monotherapy for bipolar depression	❌ Risk of switching/rapid cycling; use quetiapine, lurasidone, cariprazine, lamotrigine, Li
Agitated depression = treat with an antidepressant alone	❌ Consider **mixed features**: highest suicide risk; mood stabiliser/atypical
Grief excludes MDD	❌ Bereavement exclusion **removed** in DSM-5
Wish to "join" the deceased = suicidal MDD	❌ Normal grief phenomenon; worthlessness/guilt points to MDD
A stressor present → adjustment disorder	❌ If full MDD criteria are met → **MDD**
Irritable 12-year-old with outbursts → paediatric bipolar	❌ Chronic, non-episodic → **DMDD**
Forgetful elderly patient → dementia	⚠️ Consider **pseudodementia** (depression)
PHQ-9 high → diagnosis	❌ Screen only; doesn't assess mania; item 9 ≠ risk assessment
Mild depression → antidepressant	❌ Step 1: psychoeducation, monitoring, self-help, BA, exercise
Antidepressant "not working" at 2 weeks	❌ Effect takes **4–6 weeks**
Stop the antidepressant at remission	❌ Continue **6–9 months** (≥ 2 years if recurrent)
Baby blues → antidepressant	❌ Self-limiting ≤ 2 weeks; reassure. Psychosis → **admit**
Diagnose MDD in an active heavy drinker	❌ Reassess after abstinence
TCA for a suicidal outpatient	❌ **Most lethal in overdose**
Atypical = "not typical presentation"	❌ A specific specifier: **reactive mood**, hypersomnia, hyperphagia, leaden paralysis, rejection sensitivity
Catatonic specifier needs ≥ 2 features (slide 34)	⚠️ DSM-5-TR requires **≥ 3 of 12**. Slide inconsistency
Bipolar suicide 15–20%	⚠️ Older inpatient figure; current **6–7%** (10–30× baseline). Know both